

Surgery for bile duct cancer

Some people are able to have surgery to remove a bile duct cancer.

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About surgery for bile duct cancer

You may have surgery to remove [bile duct cancer](#). Or you may have it to help relieve symptoms caused by bile duct cancer blocking the bowel or bile duct.

Surgery to remove the cancer

For some people, surgery to remove bile duct cancer can cure the cancer.

Your surgeon will decide whether they can remove all the cancer based on different tests. If they can remove all the cancer, it is called a resectable cancer.

Surgery for bile duct cancer is a specialised treatment. You will be referred to a specialist centre, where there are surgeons with experience in doing this type of surgery. The type of operation you have will depend on:

- where in the bile ducts the cancer is
- the size of the cancer
- whether it has spread into nearby tissues.

Surgery for bile duct cancer is usually a major operation. This means you need to be well enough to be able to cope with it. Your surgeon will talk about this and discuss the benefits and risks of surgery with you. Together you can decide whether surgery is the right treatment for you.

A liver transplant is not usually an option. It may be offered, but only in rare situations. Your surgeon can discuss this with you.

In a small number of people, bile duct cancer is found at a [very early stage](#). The surgeon removes only the bile ducts containing the cancer. Then they join the remaining ducts in the liver to the small bowel. This allows the bile to flow again.

Most operations for bile duct cancer are much bigger operations and involve removing more tissue.

Types of surgery for bile duct cancer

The operation depends on the type of [bile duct cancer](#) you have.

Surgery for intrahepatic bile duct cancer

If you have intrahepatic bile duct cancer, the cancer is in bile ducts in the liver. The surgeon removes the affected part of the liver and the bile ducts.

Surgery for perihilar bile duct cancer

If you have perihilar bile duct cancer, the cancer is where the left and right hepatic ducts meet, just below the liver. The surgeon removes the bile ducts, the gallbladder and the part of the liver close to the bile ducts.

Sometimes the surgeon also removes part of the pancreas and part of the small bowel. This is the same as surgery for distal bile duct cancer.

Surgery for distal bile duct cancer

If you have distal bile duct cancer, the cancer is in bile ducts near the pancreas and small bowel. The surgeon removes the bile ducts, the gallbladder, part of the pancreas and part of the small bowel.

This is a major operation. It is only suitable for people with early-stage distal bile duct cancer. In some people, it may cure the cancer.

You may have 1 of the following operations.

Pylorus-preserving pancreato-duodenectomy (PPPD)

In this operation, the surgeon removes:

- the head of the pancreas
- most of the first part of the small bowel (duodenum)
- the common bile duct
- the gallbladder
- the surrounding [lymph nodes](#).

This operation is also called a modified Whipple's operation.

Pancreato-duodenectomy (Whipple's operation)

This is like a PPPD operation, but the surgeon removes the lower part of the stomach (the pylorus) as well. It is also called a Whipple's operation.

Whipple's operation

Image: This diagram shows the liver, gallbladder, stomach, pancreas and duodenum. The parts that are removed during a Whipple's operation are shown in white. These are the head of the pancreas, which is the part closest to the duodenum, the gallbladder, part of the common bile duct, the lower part of the stomach and the duodenum. The organs that remain after surgery are in colour. These are the liver, the upper part of the stomach, part of the common bile duct and the tail of the pancreas.

The sections in white are the parts removed during a Whipple's operation.

After a Whipple's operation

Image: This diagram shows the organs that remain after a Whipple's operation and how they are attached. The organs are the liver, the upper part of the stomach, the lower part of the common bile duct, the remaining small bowel and the tail of the pancreas. The stomach, bile duct and tail of the pancreas are all attached to the small bowel in different places.

The surgeon attaches the remaining part of the stomach, the remaining bile duct and the tail of the pancreas to the small bowel.

After your operation

You may need to stay in a high-dependency unit (HDU) or intensive care ward straight after the operation. After a few days, you will move to a general surgical ward. Most people need to be in hospital for about 10 days to 3 weeks after these types of operation.

We have more information about [recovering after surgery](#).

After surgery to the pancreas

The pancreas normally produces insulin and digestive juices. If you have had part of the pancreas removed, it will produce smaller amounts of these substances. You may need to have medicines to replace them.

Insulin injections

If you have had part of your pancreas removed, your blood sugar level may not be well controlled. This is because the remaining pancreas may not make enough insulin. You may need to have insulin injections to help control your blood sugar. This is usually only until the remaining pancreas recovers and starts making insulin again.

Digestive juices

The pancreas normally produces digestive juices (enzymes). These help your body break down and absorb fats and protein from your food. If you have had part of your pancreas removed, your body may not be able to absorb nutrients from food. This can cause symptoms such as:

- [diarrhoea](#)
- stools (poo) that float, look pale and smell bad
- difficulty putting on weight.

You may need to replace the digestive juices (enzymes) that the pancreas normally makes. This will depend on the type of operation you have had and your ongoing symptoms.

Pancreatic enzyme replacement therapy drugs

You can take capsules that replace the digestive enzymes. You usually take these each time you eat or have a milky drink. Your surgeon, nurse or a dietitian will give you more information about how to take the capsules. You will need to take them for the rest of your life.

Some commonly used enzyme replacement drugs are:

- Creon[®]
- Nutrizym[®]
- Pancrease[®]
- Pancrex[®] .

The most commonly used is Creon[®]. Most replacement enzymes are made from pork. There is no alternative that is not pork based. Talk to your healthcare team if you are concerned about this.

Creon[®] has been approved for use by Jewish people by the Chief Rabbi. The London Central Mosque has said that Muslims may use pork-based medicines if there is no alternative. If you are worried about this, speak to your religious leader.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our bile duct cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help